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**Topic: Rebuilding Trust in Healthcare: A Public Relation Strategy for Ibn
Sina Hospital, Bangladesh**

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Rebuilding Trust in Healthcare: A Public Relation Strategy for Ibn Sina Hospital, Bangladesh

Introduction:

Ibn Sina Hospital is one of the renowned health care providers in Bangladesh under the Ibn Sina Trust, whose motto/vision is to serve humanity as a whole, with the promise of affordable healthcare to the Bangladesh people. The Trust started its operation in June 1980 and it currently offers a wide range of services, such as hospital services, diagnostic services, diabetes services, pain and physiotherapy, dental services, corporate and home services, health check-up packages and child development centre. It also emphasizes the existence of ISO 9001:2015 certification, which is also reflected in its official site and emphasizes the importance of structured service quality and patient trust.

The quality policy of the hospital is evidently service based: laboratory testing is tracked through sample collection to reporting, with different collection booths, trained personnel, stringent waste-disposal policies, internal quality control, and external quality control assistance.

Background of the Research: What Made us select Ibn Sina Hospital

We selected Ibn Sina Hospital, as it is located at the crossroads of two significant realities in the healthcare environment in Bangladesh high institutional presence and low social trust. Ibn Sina is not an outlier provider; it belongs to a big, multi-branch privatised healthcare network with diagnostic centres, hospitals and special services in Dhaka and other districts. Its trust alone claims that it started in the early 1980s and position itself as a top healthcare provider offering advanced radiology, laboratory and hospital services. Simultaneously, recent news indicates that the private health sector currently has a significant portion of care delivery in Bangladesh, making a hospital such as Ibn Sina significant in terms of PR research since its reputation now influences not only a brand but overall trust in the private healthcare.

The historical context is a factor as well. Ibn Sina has been a brand that has gained popularity in the medical field and its official communication highlights on technology, specialist doctors and points of service nationwide. It is a good case study of reputation management in that an

established brand of trust is likely to be consistent in all its branches and service touch points. Reputation is a strategic asset in a market whereby the shortcomings created by the public facilities will be addressed by the private hospitals. Recent reports that indicate that in Bangladesh, more than 60 percent of healthcare services are offered by the private hospitals and the sector is also affected by weak monitoring, inequitable quality and gaps in accreditation. It is the combination of the two that makes Ibn Sina such a useful research subject. It is familiar, popular, and susceptible to trust shocks.

What the Secondary Research Indicates:

The secondary research presents a rather ambiguous yet eye-opening picture. On the bright side, the very platform that Ibn Sina uses demonstrates a wide service presence and a high technical potential, which is why a patient still opts to choose it. On the down side, the media chronicle is replete with accrument of the brand in relation to diagnostic accuracy and compliance. In 2019, Daily Sun reported a complaint of a faulty platelet test in the Dhanmondi branch, where the matter was related to the diagnosis of dengue. A similar report about a lawsuit against the chairman of Ibn Sina and others over an allegedly erroneous blood test result was reported in the Daily Star. More recently, New Age and Dhaka Tribune reported that four privately run hospitals and diagnostic centres in Cumilla, including an Ibn Sina one, were closed temporarily due to irregularities like absence of valid licenses, no price lists, unavailability of equipment or unreported dengue cases. In March 2026, BSS discovered that Narsingdi Ibn Sina General Private Hospital lacked an expired license, duty physicians and technicians, and an unsanitary setting.

These reports make a narrative problem when they are together. It may not be a single instance of fake news; instead, recurring accusations and regulatory actions can be boiled down to a bigger narrative by the population that the hospital is not a reliable one. This is how the reputational harm develops: a single complaint regarding a test result, a single shutdown to find irregularities, and one inspection result may turn into a widespread assumption that all branches are unsafe or negligent. That is more important than in most other markets since in a healthcare market, patients are not purchasing a luxury, they are purchasing confidence. This is serious, as the sector coverage of the Business Standard demonstrates: despite the growing trust in local healthcare, the professionals continue to state that the lack of proper supervision can lead to the ruin of the confidence not only in the infamous private hospitals but also in the reputable ones. Prothom Alo

also documented maternal fatalities in C-sections in various private hospitals, which adds to a wider societal concern on safety and responsibility in the private care.

Primary Research: Sentiment and Survey:

The survey data told that 51 rated Ibn Sina as Good, 24 as average and only 3 as poor. What that implies is that the feeling is not unanimously positive but rather unconditionally positive. The response pattern indicates that the people continue to use and respect the hospital but their trust is not automatic but conditional. Family referral or doctor recommendation as the initial source of awareness has also the majority of responses, which indicates that interpersonal trust continues to be stronger than advertising or social-media persuasion. Wait times are frequently recorded between 15 and 30 minutes or between 31 and 60 minutes and those above that are recorded mostly amongst a few frequent users. All in all, the mood could be classified in terms of positive, neutral, and negative with the positive being clearly dominant, the neutral substantial, and the negative limited yet meaningful.

Service level responses are also important. The sheet has quite positive scores in terms of registration, staff conduct, doctor interaction, cleanliness, and service provision, yet ambivalent scores in terms of costs and expenses, emergency care, billing clarity, privacy, and waiting comfort. This trend is important since it informs us on what people actually consider in a hospital not only the quality of the treatment, but also the quality of the process. The modern popular attitude is not thus Ibn Sina is bad; rather it is more like Ibn Sina is handy, yet it has to demonstrate consistency, transparency and compliance. It is a typical PR dilemma, since a good brand promise must now be fulfilled by operational demonstrations.

What We Decide and Why We Desire to Take Action:

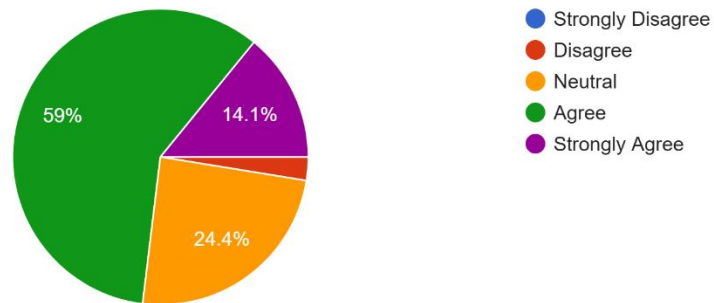
We find that Ibn Sina is both a strategically significant yet reputation-sensitive body. It is not rejected by the masses. Rather, the evidence seems to be that people are having two concepts simultaneously: they know its size, its history and its capacity to serve, but they are also concerned over the accuracy of the diagnosis, compliance and consistency at the branch level. This is the very reason why we chose Ibn Sina to research. It provides us with a real-life example of media coverage, public opinion, regulatory intervention, and customer experience interacting. The program must thus be aimed at repairing trust: open quality assurance, more robust branch-level

standards, transparent communication to the populace, quick correction of mistakes, and an active reaction to fake news. Reputation management is not cosmetic in an industry where trust is the product; it is a component of patient safety.

Here is the survey graph:

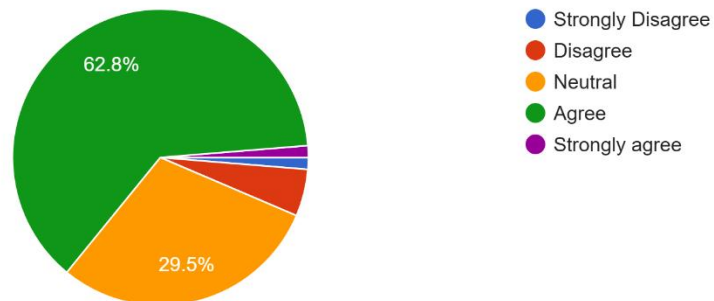
The doctors listen carefully to patient problems.

78 responses



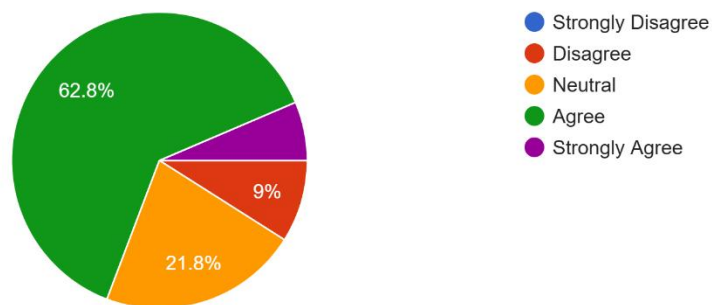
Overall, I am satisfied with the services of Ibn Sina Hospital.

78 responses



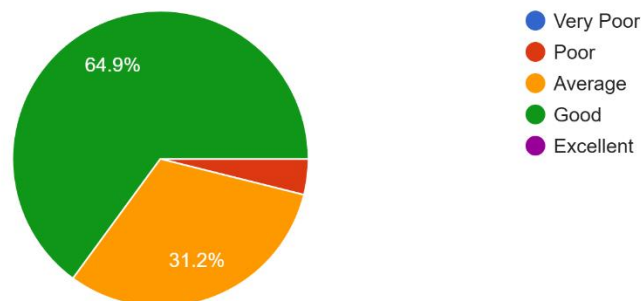
I would recommend Ibn Sina Hospital to others.

78 responses



How would you rate Ibn Sina Hospital overall?

77 responses



Ibn Sina Analysis of Situation:

The strength of Ibn Sina, as a PR case, lies in being a legacy healthcare brand and a service organization that is sensitive to trust. The trust claims that it started its operations in June 1980 with the vision of serving humanity at affordable cost and it continues to focus on diagnosis, consultation, hospital care and presence in multi-branches in Dhaka and other cities. That provides it with brand recognition, though it also increases expectations with this well-known of a name, a single grievance can go a long way.

Reputational risk is the most vital situational pressure. Recent news coverage seems to repeatedly relate Ibn Sina or the broader private-care industry with compliance and quality issues. The Daily Star covered a lawsuit regarding a supposedly incorrect dengue blood test result; Prothom Alo covered maternal deaths in C-section cases in various hospitals, including Ibn Sina Hospital in Sylhet; and New age and Dhaka Tribune covered that an Ibn Sina had temporarily closed due to issues with a missing license, no technician, expired reagents, and non-reporting dengue cases. Individually, the government and DGHS have been driving nationwide inspections and licensing reform due to the weakness of the regulation of the privately-owned facilities.

SWOT:

Strengths: The biggest asset of Ibn Sina is a time-earned trust. Since 1980, it has been in operation, poses as inexpensive, and is recognized as offering diagnostic and investigative services, which are very pertinent within the outpatient-centred healthcare market of Bangladesh. Its physical presence and accessibility, which is by hotline also render it easily accessible, also predisposes it to be approached as an approachable, everyday hospital versus a strictly high-end hospital.

Weaknesses: Inequality of perceived quality is the largest weakness. Recent reports on irregularities and previous allegations involving tests and maternal care expose the brand to the one bad branch effect of the entire brand. Besides this, there is also survey data that indicates people still like the hospital, though not without criticism: a significant number of the people in the survey are in the Good to Average category and do not say that they are unconditionally loyal to the hospital, thus being satisfied but weakly attached.

Opportunities: The size of the private health sector in Bangladesh is large and continues to grow, and government policy is changing to more organized licensing and accountability. That leaves a gap that Ibn Sina can use to be an obedient, transparent, patient-centered private brand. The market also has a larger market share due to the instability of regional medical travel. Reuters indicated a decline in Indian medical visas to Bangladeshis and an increase in space to China, and another report indicated the inflow of Bangladeshi patients to Kolkata falling and partially recovering. Part of that gap in trust can be captured by a domestic hospital that can convey quality better.

Threats: The big threats are increased regulation, accelerated public scrutiny, and increased competitors. Evercare advertises as a 425-bed, JCI-accredited super-specialty hospital; Square

boasts 700 beds, 3,000 employees, online booking and payment options, and excellent support services; Popular mentions online booking and rate-chart access. In comparison, accessibility and diagnostic trust is the greatest strength of Ibn Sina, as opposed to luxury positioning, hence any quality service failure can soon drive patients to the rivals with more conspicuous quality cues.

Audience Segmentation:

Ibn Sina has a core readership of urban, working-age, and family-oriented, demographics. In 78 responses, mostly Dhaka-based, and aged 18–45 years, indicates that patients, attendants and guardians are all important players. This implies that this choice of hospital is not only influenced by patients, but also by family members and companions. Family, doctors, and friends are the primary sources of discovery, with less contribution of the digital channels.

The audience is psycho graphically risk-averse, reassurance-seeking. The patients do not only appreciate the outcomes of the treatment, but also appraise such aspects of the treatment as the communication with the doctor, the attitude of the staff, the cleanliness, the waiting period, the openness of the billing and the privacy. This implies that the reputation is very much reliant on apparent service quality, as opposed to clinical assertions.

An urban-rural gap is evident. Whereas the urban populations can do so via the digital platforms, such as Facebook and YouTube, the rural and semi-urban populations depend on referral, physical presence, and word of mouth. This necessitates a two-way communication strategy with cities and community-based trust-building beyond the city.

A major opportunity is generationally, younger audiences. Bangladesh has a huge number of young people, which means that younger users will have a higher tendency to investigate the hospitals online and demand quicker and more open services, whereas older generations will rely on personal recommendations. Ibn Sina is thus faced with a need to strike a balance between digital credibility and good offline relationships.

Some of the main Areas of Concern:

The focal problem is trusting repair. The brand should decrease the disconnect between the legacy identity and the present public-risk profile. The second is the problem of operational transparency: waiting time, clarity in billing, the accuracy of the tests, compliance at the branch level should be

transparent and regular. The third problem is audience-specific communication, as rural referral-based users and urban digital users do not act in the same way. The most promising opportunity is to make compliance, service quality, and diagnostic credibility a quantifiable reputation strategy. Practically, Ibn Sina does not just need to defend itself, but it must demonstrate its reliability at each point of contact.

Methodology

In this research, the PR research design was mixed-method to learn about the reputation of Ibn Sina as perceived by the population and as expressed by the media. The design incorporated both quantitative survey research and qualitative content analysis and reviewing online and offline discussion patterns. This was a correct method since hospital reputation cannot be developed by a single source; rather it is created by direct experience, media framing, peer recommendation and societal trust in the performance of institutions.

Sampling

As the main quantitative element, we selected a non-probability convenience sample of 78 respondents, who were chosen based on available contacts and campus/community contacts. This sample is convenient to student-level research that is exploratory in nature, though not purported to be representative nationally. The sample size consisted mainly of people who had some knowledge about the private healthcare services in Bangladesh, and many of these people might have been the potential users, patients or attendants of the patients. Since Bangladesh has a high number of young people and a digitally connected urban population, this sample can help comprehend how the younger and more exposed populations perceive healthcare brands. Recent national and international reporting indicates that the working-age population is high in Bangladesh and young adult population is high in Bangladesh and this has made these young adults a strategic audience in terms of their communication in hospitals.

As a supplement to the survey, we employed purposive sampling of secondary material: recent news items, institutional web pages, regulatory sources, and popular discussion about Ibn Sina and the overall private healthcare market. This has incorporated media coverage of compliance, testing, licensing, and quality of service, since these are the areas which are most likely to influence hospital reputation.

Instruments

A structured questionnaire consisting of mostly closed-ended questions, and several open-ended prompts was the primary tool of primary data collection. It assessed knowledge about Ibn Sina, perceived service quality, trust, cleanliness, staff behaviour, waiting time, pricing, and chances of being recommended. The use of the closed ended items resulted in the generation of simple descriptive statistics whereas the use of the open-ended responses was able to generate common themes like trust, delay and transparency.

A news and social discussion content-analysis checklist was also utilized with us. This checklist coded the positive, neutral or negative; whether the source was about service quality, compliance, diagnostics, or crisis; and the framing was informational, critical or reputationally damaging. To conduct the observational section, we perused above-ground online responses and commentary trends, focusing on recurring issues instead of individual statements.

Data Analysis

The qualitative data were processed with the help of the simple descriptive statistics: the number of frequencies, percentage, and simple trend reading. It is appropriate when having a sample of 78 since the aim was reputation mapping and not hypothesis testing. The thematic analysis of the qualitative responses was conducted. The similar notions were clustered into the themes like trust, speed of service, staff behaviour, price sensitivity, and fear of misinformation. The survey results were then compared with the media content to determine where the experience of the people was reflected or disparate to the media accounts.

Limitations

The research has a number of limitations. To start with, the survey is based on convenience sample, meaning that the findings are not applicable to all hospital users in Bangladesh. Second, hospital reputation is a sensitive issue and as such some of the respondents might have provided guarded or desirability responses. Third, there is unequal access to digital in Bangladesh: recent BBS-based reporting demonstrates that there is a high urban-rural digital divide, with rural internet penetration significantly lower than urban penetration, indicating that online opinion can be more readily secured in urban than rural settings.

Fourth, the regulatory and service conditions in the private healthcare may vary rapidly. National reporting has indicated persistent issues with licensing, inspection and compliance in private hospitals, indicating that the findings may be obsolete in case of a change in the institutional setting.

Ethical Considerations

The research was conducted in accordance with the principles of the human-subject ethics. The purpose of the survey was explained to the respondents and participation in the survey was voluntary, and no personal identifiers were reported in the final analysis. The guidelines of Bangladesh Medical Research Council focus on informed consent, understanding of the participant, and ethics review of those studies that involved human participants.

Online material was also handled with caution. They would only observe thematically using public posts and comments but not identify them. No personal or confidential source of information was gathered. This is relevant since the discussion on the internet about hospitals is usually accompanied with anxiety, anger, or misinformation, and such information must be approached with moderation.

Bangladesh Research challenges

There are a number of practical challenges to conducting research in Bangladesh. Access is not uniform. Urban respondents are more accessible than rural since internet use and connectivity are significantly greater in urban as opposed to rural regions. Another problem is trust: in healthcare research, not all respondents are always willing to openly criticize a familiar hospital, and they are afraid of the consequences of the future services. The resource constraint is also important, as in many cases student research is time-constrained, has limited field funds, and has limited capability to perform extensive probability sampling.

Situation Summary

Ibn Sina is an established and popular brand of private healthcare, with a history of affordability and service since 1980. Nevertheless, its present reputation portrays a balancing act between the old faith and new issues of credibility. Since the role of the private form of healthcare in

Bangladesh is significant, there is a high level of expectations among the population, and any form of perceived lapse receives immediate publicity.

Studies have indicated that individuals do not rebuff the brand but take it with a grain of salt. It is perceived as something available and common sense, but recurrent media coverage - the dengue test scandal and shutdowns due to compliance- have established a disconnect between official rhetoric and social reality. These are the events that have not erased the trust but made it more vulnerable.

The results of the survey support this trend: the sentiment is mostly positive yet not entirely confident. Patients appreciate access and acknowledgment yet demand enhanced transparency, communication and consistency between branches. The sentiment of the masses is not, however, hostile, but suspicious. The temper of the people is not, however, aggressive, but suspicious.

This establishes a strategic course of action: the greatest opportunity is to transform reserved trust to robust confidence by demonstrating quality assurance, enhanced communication, and aggressively managing misinformation. The key threat is that any subsequent event may serve to strengthen the suspicion and undermine credibility even more.

SMART Objectives

1. Increase Positive Sentient

Young urban youth increase positive brand value within 12 months. The way to do it by survey and social media monitoring.

2. Reduce Negative mentions

Reduce negative media and social references of fake report, irregularity, and license issue by 30% after 9 months using key word monitoring and content analysis.

3. Improve the level of trust in diagnostics and transparency

Improving reliability in diagnostic transparency within 6 months by 20% to measure through patient survey and using feedback form.

4. Reduce service complain

Lower complaints about waiting time and internal miss management by 20% within 9 months. To track down they can go through complain logs and feedback of the patients.

5. 90% rapid misinformation response in 12 months

Within 12 months, make sure that 90 percent of confirmed cases of misinformation are admitted within 2 hours and corrected within 24 hours, and this is monitored by a response log.

Target Audiences

Core target: urban patients and attendants aged between 18 and 35 in Dhaka, and other mega cities. This is the group that should be given the highest priority since Ibn Sina has a good branch presence in Dhaka and most of the other urban centres and our survey already contains a high proportion of Dhaka city respondents and people of working age. They are digitally conscious, shop hospitals prior to visiting and evaluate the brand in terms of speed, employee conduct, transparency and online reputations. Many respondents indicated that they initially learned about Ibn Sina via family, friends, doctors, social media, and this reveals that this segment is affected by digital and interpersonal trust.

Secondary audience: family decision-makers, caregivers, and older users between the age of 36 and 60 and above. Hospital choice in Bangladesh is not an individual one, particularly in surgery, maternity, diagnostics and management of chronic diseases, but rather, a family choice. This consumer group places more emphasis on doctor recommendation, visible cleanliness and clear billing than branding. The answers in our surveys indicate that we rely on family and physician referrals repeatedly, which implies that Ibn Sina needs to convey the trust in a manner that will convince guardians not patients only.

Tertiary audience: semi-urban and rural users, particularly referral-based patients. This population is significant since a significant urban-rural digital divide exists in Bangladesh: 75.7% of urban residents use the internet, whereas the corresponding figure is 43.6% in towns. It implies that not a significant proportion of rural and semi-urban patients will be reached effectively solely by using social media, as they are more dependent on the word of mouth, local physicians, signage, and experience of service in the branches. Offline trust-building and referral credibility then should not be an afterthought rather a strategy of the audience as Ibn Sina should treat them.

Influencer segment: physicians, diagnostic referrers and local opinion leaders. In the case of healthcare, perception is formed by these actors prior to the patient going to the hospital. Since Ibn Sina has a broad network of diagnoses and consultation in different branches, this audience is imperative to maintaining the footfall and reputation. Practically speaking, the brand needs to be successful among the young urban users online, family decision-makers offline, and referrers in terms of consistency and compliance.

Core messages

1) Competent care, available to Bangladeshi families

This message must pre-empt the legacy, affordability and service focus of Ibn Sina. It must be written in simple Bengali and understanding language, as choice of hospital is usually a family decision in Bangladesh not a consumer decision. The message ought to assure the patients that there is Ibn Sina who is reliable to deliver care without any undue complexities and snobbism.

2) First comes your safety and accuracy of the test

This is the most critical corrective message since the reputational harm is largely due to diagnostic and compliance issues. The voice must be factual rather than defensive: discuss quality controls, internal review procedures, physician supervision and dealing with complaints. The organization must not evade the matter since media coverage has already linked the brand to a wrong-test claim and then to anomalies in one of its privately owned facilities, the organization ought to make it apparent.

3) Excellent, honest service in all branches

According to our survey results, users consider Ibn Sina based on treatment, but also on waiting time, staff behaviour, communication, cleanliness and transparency in billing. This message must then focus on consistency in service delivery across the branches, transparent rates, and courtesy towards patients and attendants. This is particularly essential since the trust has several units in Dhaka and beyond Dhaka and therefore uniformity is as essential as quality in the head branches.

4) Not a city-only hospital, a countrywide one

This message ought to associate Ibn Sina with national service and larger social value. It must address the urban patients, the suburban families and the referral-based users, but also recognize

that not all of Bangladesh has the same access to digital. Due to the high usage of internet in urban and low in rural areas, there should be alternative channels of delivery of the same message to different audiences.

Strategy

The general approach would be reputation restoration via evidence-based communication. It implies that Ibn Sina will no longer need mere promotion but a three-step model of communication: demonstrate quality, describe processes, and act promptly. It is not a cosmetic image campaign; it is a trust-restoration program. The brand ought to post more material of standards, reveal its service processes, and establish a strict system of disciplinary response to misinformation, complaints, and services failure. That strategy fits the present-day environment in Bangladesh, where young, connected audiences demand transparency, and older and rural audiences continue to be greatly dependent on doctor and family referrals.

Community engagement should be a second strategy pillar. Since patients in Bangladesh tend to trust individuals prior to the institutions, Ibn Sina must develop social proof with doctors, cured patients, community health initiatives, and local collaborations. Thought leadership should be a third pillar: the hospital must also be heard on diagnostic standards and preventive healthcare, dengue awareness and maternal health, as well as patient safety, so the brand is not perceived as a service provider, but a responsible health actor.

Tactics and channels

In the case of earned media, Ibn Sina needs to conduct regular press conferences, react swiftly on accusations, and place the commentary of experts in health and business media. The credibility can be enhanced by inviting journalists to see the real processes of service, rather than events, which will help avoid the growth of minor problems.

In the case of the owned media, the hospital needs to improve its web and social media sites with detailed branch-specific information, physician descriptions, test descriptions, and contact centres. The accessibility will be enhanced by short videos in Bengali with common issues tests, reports, privacy, emergencies which will help the mixed digital audiences.

In the case of paid media, Ibn Sina can target Facebook and YouTube advertisements geographically around the branches and target seasonal health campaigns such as dengue awareness and maternal care. The use of messages should not be too promotional but informative and trust-building.

In the case of events and CSR, community trust can be established through activities like free health camps, doctor question and answer, blood donations, and reaching out to students. Messaging that is based on festivals (Eid, monsoon, national events) can assist to relate the brand to local culture and daily health requirements.

In the case of government and stakeholder relations, it is vital to organize the communication with DGHS, local health authorities and administrators, particularly in the case of tightening of the regulation. It is also essential to strengthen referral networks among doctors and community leaders since trust in Bangladesh tends to go through personal relations.

Timeline:

Activity/Phase	Month 1-2	Month 3-5	Month 6-9	Month 10-12	Key Deliverables
Research & Sentiment Audit	✓				Baseline data, insights
Internal Service Audit	✓				Gap identification
Messaging Framework Development	✓	✓			Unified key messages
Staff Training (Communication)	✓	✓			Improved patient interaction
Website & Digital upgrade		✓	✓		Updated online presence

Transparency campaign		✓	✓		Trust building content –
Media Relations (Press, PR)		✓	✓	✓	Media visibility
Patient Feedback system		✓	✓		Real-time feedback
CSR & Health Camps			✓	✓	Community engagement
Digital Ads & campaigns			✓	✓	Youth engagement
Influencer Partnerships			✓	✓	Social Credibility
Festival-Based Campaigns			✓	✓	Culture Relevance
Rural Outreach Programme			✓	✓	Offline trust
Impact Evaluation & Reporting				✓	Final performance report

Budget Estimate:

Cost Component	Low Budget (8-12)	Medium Budget (20-30 lakh)	High Budget (50- 70 lakh)
Social Media Management	2-3 lakh	5-7 lakh	10-15 lakh
Content Creation (Video/Design)	2 lakhs	4-5 lakh	8-10 lakh

Paid Advertising (FB, YouTube)	2-3 lakh	6-8 lakh	15- 20 lakh
CSR Event & Health Camps	1-2 lakh	3-4 lakh	8-10 lakh
Influence Marketing	-----	2-3 lakh	5-8 lakh
PR & Media Relations	-----	2-3 lakh	10-15 lakh
Monitoring & Research Tools	0.5- 1 lakh	1-2 lakh	3-5 lakh
Website Upgrade	Included basic	Included improved	Advance system
Total Estimated Cost	8-12 lakh	20-30 lakh	50-70 lakh

Evaluation & Measurement

Ibn Sina requires a solid assessment system since the main aim is to restore trust, rather than to raise visibility.

1. Key performance Indicators

KPIs Reputation & Sentiment are concerned with how people think of them. The first indicator is the growth of positive sentiment, and the goal is to increase positive views among young audiences in cities by approximately 25% in 12 months. Meanwhile, negative sentiment reduction, particularly when it comes to matters such as incorrect test results, anomalies or bad customer service, must drop by at least 30 percent. The Net Reputation Score (NRS) can be derived by taking the difference between negative and positive mentions and dividing them by the total mentions. Moreover, confidence in diagnostics, billing transparency, and overall care will be measured by a Patient Trust Index which will be obtained based on the follow-up survey.

Media KPIs assess the coverage of Ibn Sina in the news and in society. These are the amount of the media coverage, coverage tone (positive/neutral/negative ratio), and share of voice in comparison with other competitors like Square or Evercare. Crisis response time is another

important KPI, and the target is to recognize the problems within 2 hours and provide comprehensive clarification within 24 hours.

Digital Engagement KPIs are online performance indicators. These are engagement rate (likes, comments, shares to reach), follower growth, video completion rates, and campaign click-through rates (CTR). It is also important that the response time to patient inquiries on Facebook or other platforms is as quick as possible which has a direct impact on trust.

Service Experience KPIs relate the PR results to the actual hospital performance. They are patient satisfaction scores, rate of reduction of complaints, average waiting time and repeat patient rate. As our survey already demonstrates ambivalent levels of satisfaction, growth in these measures will be a direct indicator of the effectiveness of PR.

2. Tools for Measurement

A combination of both international and local tools is to be utilized.

In terms of media monitoring, sites such as Meltwater or even local clipping agencies (such as BAC-type agencies) can monitor news in the press as well as TV and radio channels such as Prothom Alo, Daily Star, and TV.

In the case of digital analytics, Facebook Insights, YouTube Analytics, and Google Analytics are some of the tools that offer real-time feedback on audience behaviour, engagement and reach.

To collect surveys and feedback, it is possible to rely on Google Forms to conduct pre- and post-campaign surveys, and patient experiences can be captured by in-hospital digital kiosks or SMS feedback systems once the services are provided.

3. Methods of Evaluation

The most direct method is pre–post survey comparison. The current 78-response survey is used as the baseline. Changes in trust, satisfaction, and recommendation likelihood will be assessed using follow-up surveys at 12 months.

The content and sentiment analysis are to be performed monthly. Media coverage and social media remarks will be sorted into positive, neutral and negative themes. Particular focus needs to be put on such recurrent issues as diagnostic accuracy or delays in service.

Trend analysis will be used to monitor whether negative stories are decreasing as time goes by and positive stories are getting higher. This is to determine whether communication efforts are effective or require modification.

It is also essential to integrate operational data. PR metrics should be analysed with complaint logs, waiting times, and service records to determine that better communication is one with actual service improvements.

Conclusion

In this study, the researcher demonstrates that Ibn Sina is still a renowned and a popular private healthcare provider in Bangladesh, yet its image is now formed as a paradox of the old-fashioned trust and new anxieties.

The discussion supports the argument that trust is the focal point. Even in an isolated case, in a competitive private healthcare setting, when patients are so dependent on the personal experience, and the word of mouth, the overall opinion of the population can be affected. Simultaneously, the varied audience of Bangladesh, the digital user in the city and the referral patient in the rural areas, needs targeted communication approaches.

The PR strategy suggested will focus on reputation restoration by way of transparency, better service experience, and proactive communication. With operational improvements and an effective combination of improved communications and quantifiable goals, Ibn Sina can rebuild confidence over time.

To sum it up the future reputation of the hospital will be based not just on what it says, but it will rely on whether it can provide quality care on a regular basis. Visibly performing will be more likely to bring sustained trust than promises.

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